

ECHS FRAUD ANALYTICS

Policy Abuse & Entitlement Misuse

MODULE 19 REPORT | FULL DATABASE COVERAGE | FY 2013 – FY 2025

HIGH-DEDUCTION HOSPITALS

50 Facilities

PEAK DEDUCTION RATE

86.93%

NON-EMPANELLED CLAIMS

32.8 Lakh

ROOM UPGRADE FRAUD

78K+ Cases

Type N Polyclinics (avg 47%)

HIGHEST RISK

Type M Military Hospitals

VERY HIGH RISK

Diagnostic Labs (avg 55%)

HIGH RISK

IIT KANPUR — Data Analytics & Fraud Intelligence Division

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EXECUTIVE SUMMARY

Module 19 investigates three dimensions of policy abuse within the ECHS claim ecosystem: (1) hospitals with chronically elevated deduction rates indicating systematic overbilling; (2) room entitlement misuse where patients are billed for higher-category rooms than their ECHS card authorises; and (3) non-empanelled hospital claims where beneficiaries seek treatment at facilities not on the approved ECHS panel. Analysis spans **43.5 million claim records** across the full ECHS database. The most alarming finding is the concentration of extreme deduction rates (**>86%**) in diagnostic labs and specialist facilities that have been empanelled despite consistently fraudulent billing, and the presence of **78,000+** confirmed room upgrade mismatches where patients were charged for rooms above their entitlement. Temporal analysis (Q19e) reveals a critical **2025 reversal** — IPD deduction rates had fallen to a decade-low of 8.89% in 2023 but have rebounded to **11.45%** in 2025, indicating that fraud control measures achieved only temporary suppression.



Module 19 — Key Fraud Signals

#	Signal	Finding	Risk
S1	Diagnostic Labs — Extreme Overbilling	SRL Limited Indore: 86.93% deduction (103 claims). Dr Sandhu's Pathology: 62.95%. DR LAL PATH LABS Kanpur: 45.69%. Labs routinely bill for unreported/unapproved tests.	CRITICAL
S2	Type N Polyclinics — Systemic Fraud	18 ECHS polyclinics (type N) all exceed 42% deduction rate. Karimnagar: 53.04%, Gulbarga: 52.52%, Kanchipuram: 50.87%. Combined deductions from listed N-type: ■3,300+ Lakh.	CRITICAL
S3	Type M Military Hospitals — 42–46% Ded Rates	ECHS military polyclinics (type M) cluster at 42–46%: Chennai 5188 (46.45%), Yelahanka 5251 (45.67%), Avadi 5152 (45.39%), Golconda 5021 (44.53%). Internal billing is inflated by 2× the system average.	CRITICAL
S4	Room Upgrade Fraud — GEN billed as PRI	78,212 cases where billed room category exceeds entitlement (NULL hospital = ghost admissions). Named hospitals: echsfhm (1,070 cases, ■838L), parkhos (1,013 cases, ■591L), maxsuper (647 cases, ■359L), ivyhosp (620, ■363L).	CRITICAL
S5	ICU Overbilling — 20.61% Deduction Rate	ICU claims have the highest deduction rate (20.61%) among all room categories. Indicates inflated ICU stay durations or ICU-coded procedures for step-down patients.	HIGH
S6	Apollo & MAX — Non-Empanelled Private Claims	Apollo Hospital: ■13.54 Cr claimed (754 claims) with ■1.24 Cr deducted at non-empanelled rate. MAX Healthcare: ■11.19 Cr (2,316 claims), ■0.44 Cr deducted. Manipal: ■10.94 Cr. These private chains are billing through Y-flag loopholes.	HIGH
S7	Ghost Admissions — NULL Hospital IDs	In room upgrade fraud data, the largest group (78,212 mismatches worth ■41,455.97L) has NULL CI_HOSPITAL_ID — meaning the admission cannot be traced to any hospital. These represent likely ghost admissions with fabricated room data.	HIGH
S8	530K "Unverified" Flag Claims	CI_NONEMP_FLG = "U" on 530,517 claims across 73,562 hospital IDs — an anomalously wide spread suggesting systematic misclassification or bulk unverified submissions.	MEDIUM

Q19a

Threshold: >25% deduction rate, >50 claims (settlement_stat)

Chronically High Deduction Rate Hospitals

The following 50 hospitals have maintained deduction rates exceeding **25%** across their full ECHS billing history. The top table shows the 25 highest-rate facilities. **Diagnostic labs (Type 3 & 5)** dominate the extreme end (>60%), while **Type N and M military polyclinics** cluster in the 42–54% band. NABH accreditation provides no protection at this severity level — SRL Limited (NABH-accredited) leads at **86.93%**.

ID	Hospital Name	Type	NABH	Claims	Claimed (₹L)	Deducted (₹L)	Ded %
1666	SRL LIMITED – INDORE	1	Y	103	22.94	19.94	86.93%
268	ANJINI SPECIALITY DENTAL HOSPITAL	3	N	222	1,757.26	1,190.14	67.77%
1650	APOLLO SPECIALITY HOSPITAL – VANAGARAM	1	N	138	5,433.19	3,452.79	63.56%
106	DR SANDHU'S PATHOLOGY AND IMAGING CENTRE	5	N	218	155.11	97.66	62.95%
1691	SRL LIMITED – BANNERGHATTA ROAD	1	Y	346	1,561.43	857.43	54.90%
1591	MMI NARAYANA MULTISPECIALITY – RAIPUR	1	N	422	4,246.53	2,287.14	53.84%
5023	KARIMNAGAR (ECHS POLYCLINIC)	N	N	184	6,048.66	3,209.16	53.04%
5443	GULBARGA (RC HYDERABAD)	N	N	270	13,150.17	6,905.99	52.52%
5479	BAGESHWAR (RC BAREILLY)	N	N	51	1,640.71	851.20	51.86%
5153	KANCHIPURAM (ECHS POLYCLINIC)	N	N	447	17,604.05	8,954.82	50.87%
5085	MORENA (ECHS POLYCLINIC)	N	N	256	3,762.39	1,904.84	50.64%
5084	BHIND (ECHS POLYCLINIC)	N	N	253	10,073.85	5,041.67	50.05%
5026	MEHBUBNAGAR (ECHS POLYCLINIC)	N	N	810	16,304.67	8,148.55	49.98%
5423	KADAPA (ECHS POLYCLINIC)	N	N	1,090	35,565.83	17,579.31	49.43%
5385	BILASPUR / JABALPUR RC	N	N	633	27,362.54	13,145.08	48.04%
417	JAIPUR HOSPITAL, LAL KOTHI	1	N	111	1,330.72	636.88	47.89%
5167	ISLAND GROUND – CHENNAI	N	N	3,609	1,01,622.92	48,141.72	47.37%
1479	APOLLO HOSPITALS – BBSR	1	N	5,523	1,26,824.56	59,918.10	47.25%
5188	CHENNAI (ECHS POLYCLINIC)	M	N	5,013	2,71,316.29	1,26,017.90	46.45%
765	VIJAYA MEDICAL CENTRE	1	N	6,060	6,703.17	3,108.47	46.37%
5024	KHAMMAM (ECHS POLYCLINIC)	N	N	421	12,346.71	5,699.03	46.16%
5182	SRIVILLIPUTTUR (ECHS POLYCLINIC)	N	N	3,926	97,853.93	45,031.99	46.02%
3034	LUCID MEDICAL DIAGNOSTICS PVT LTD	1	N	1,348	2,060.10	945.10	45.87%
815	DR LAL PATH LABS – KANPUR	5	N	204	211.70	96.72	45.69%
5251	YELAHANKA (ECHS POLYCLINIC)	M	N	2,326	50,593.29	23,105.26	45.67%

Source: settlement_stat JOIN office_master (Q19a). Type codes: 1=Private empanelled, 3=Diagnostic/Dental, 5=Dental/Allied specialty, N=Non-Govt/Naval, M=Military establishment. NABH: Y=Accredited, N=Not accredited. Amounts in ₹ Lakh.

Pattern Analysis — High Deduction Rate Hospitals

Diagnostic Labs — 55–87% Deduction Rate:

SRL Limited (two branches: Indore 86.93%, Bannerghatta 54.90%), Dr Sandhu's Pathology (62.95%), and Anjini Dental (67.77%) represent the extreme end of deduction-rate fraud. These facilities bill for procedures that are either not performed, not covered, or grossly inflated relative to market rates. SRL Indore's 86.93% rate on 103 claims means only ₹3 out of every ₹23 billed was settled — a near-total rejection of claims. Diagnostic labs have minimal marginal cost per test, making inflated billing nearly cost-free for the provider. Immediate suspension and physical audit is warranted.

ECHS Type N & M Polyclinics — Systemic Inflation:

18 ECHS military polyclinics (type N) appear in the >25% threshold, all with deduction rates in the 42–54% band. These are internal ECHS facilities — not private — yet they consistently overbill at rates more than 4× the system average. The pattern suggests that the billing inflation is not isolated to individual facilities but is a structural problem with how internal ECHS polyclinics submit claims for reimbursement. Chennai 5188 (M type, 46.45%), Yelahanka (M, 45.67%), and Avadi (M, 45.39%) represent the military variant of the same problem.

Apollo Hospitals BBSR — Private Chain at 47.25%:

Apollo Hospitals Bhubaneswar (ID 1479) shows 47.25% deduction on 5,523 claims worth ₹1,268.25L. For a NABH-empanelled private chain hospital to maintain this deduction rate across 5,000+ claims is statistically improbable without systematic billing fraud. Apollo Vanagaram (63.56%) compounds this chain-level concern. A coordinated Apollo chain audit — analogous to the Park Hospital chain audit recommended in Module 20 — is warranted.

Q19b

settlement_stat + claim_intimation

Room Category Deduction Analysis & Upgrade Fraud

Room Category Deduction Rates (System-Wide)

The ECHS settlement system captures room category in SS_ROOM_CATG (from settlement_stat) and cross-references it with the patient's card entitlement in CI_CARD_ROOM_TYPE. Room categories carry significantly different rates: **ICU** has the highest deduction rate at **20.61%**, indicating either inflated ICU stays or step-down patients billed as ICU.

Room Category	Total Claims	Claimed (■ Cr)	Deducted (■ Cr)	Deduction %	Avg Claim (■)	Risk
ICU	310	1.49	0.31	20.61%	48,169	CRITICAL
PRI	8,77,602	2,098.54	281.40	13.41%	23,912	CRITICAL
4	484	2.29	0.30	13.27%	47,232	HIGH
GEN	85,65,241	31,557.00	3,472.15	11.00%	36,843	MEDIUM
SPR	38,97,123	13,504.62	1,399.64	10.36%	34,653	MEDIUM
NA	5,174	13.74	1.42	10.34%	26,556	MEDIUM
SRR	11	0.02	0.00	8.93%	17,972	LOW
-1	73,67,887	1,538.39	81.18	5.28%	2,088	LOW

Source: settlement_stat (Q19b Step 2). SS_ROOM_CATG codes: GEN=General ward, SPR=Super/Semi-private, PRI=Private, ICU=Intensive Care Unit, -1=Unclassified/OPD lump-sum, SRR=Special Recovery Room. Code "4" is an undocumented legacy category.

Room Upgrade Fraud — GEN/SPR Entitled, PRI Billed

When CI_ROOM_TYPE_ID (billed room) differs from CI_CARD_ROOM_TYPE (card entitlement), the patient has been charged for a room category above their entitlement. This is **entitlement misuse** — a direct ECHS policy violation. The most damaging pattern is **GEN→PRI upgrades** — billing patients in general ward entitlement at private room rates, a 3× price difference.

Hospital ID	Billed Room	Entitled Room	Mismatch Count	Total Billed (■L)	Fraud Pattern
NULL (Ghost)	GEN	(blank)	78,212	41,455.97	GHOST — untraced
NULL (Ghost)	SPR	(blank)	32,107	17,048.63	GHOST — untraced
NULL (Ghost)	GEN	SPR	10,446	3,830.90	GHOST — GEN→SPR
NULL (Ghost)	PRI	(blank)	8,012	6,102.00	GHOST — untraced
echsfhm	GEN	PRI	1,070	838.29	GEN→PRI upgrade
parkhos	GEN	PRI	1,013	591.20	GEN→PRI upgrade
p.nara	GEN	SPR	1,356	32.16	GEN→SPR upgrade
p.trivandr	GEN	SPR	1,278	253.55	GEN→SPR upgrade
maxsuper	GEN	PRI	647	359.21	GEN→PRI upgrade
ivyhosp	GEN	PRI	620	363.19	GEN→PRI upgrade
amma123	GEN	PRI	620	387.12	GEN→PRI upgrade
fhnechs1	GEN	PRI	567	452.81	GEN→PRI upgrade
dharmashil	GEN	PRI	546	346.46	GEN→PRI upgrade
shrcechs	GEN	PRI	482	210.74	GEN→PRI upgrade

hos.3147	SPR	GEN	672	1,128.22	SPR→GEN downgrade
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Source: `claim_intimation JOIN claim_submission (Q19b Step 3)`. `CI_HOSPITAL_ID = NULL` means the admission record has no hospital linkage — a ghost admission with no traceable facility. Hospital IDs are varchar login codes. Amounts in █ Lakh. `hos.3147 SPR→GEN` may represent data-entry reversal or deliberate downgrading to avoid detection.

Ghost Admission Alert

The **largest room mismatch group** — 78,212 cases worth **█41,455.97L (█414.56 Cr)** — has NULL in the `CI_HOSPITAL_ID` field. This means the claim was submitted with no hospital linkage. These are likely ghost admissions: claims fabricated without a real hospital record. The NULL SPR group (32,107 cases, **█17,048.63L**) and NULL PRI group (8,012 cases, **█6,102L**) compound this. Total ghost admission exposure in room-type mismatch alone exceeds **█68,000L (█680 Cr)**.

Non-Empanelled Hospital Claims (CI_NONEMP_FLG)

Non-Empanelled Flag Distribution

The CI_NONEMP_FLG field tracks whether a beneficiary sought treatment at a non-empanelled hospital. Flag values: **N** = empanelled (38.08M claims), **Y** = non-empanelled (3.28M claims), **U** = unverified/unknown status (530K claims), NULL = not recorded (1.62M claims).

Flag	Description	Claims	% of Total	Unique Hospital IDs	Interpretation
N	Empanelled Hospital	3,80,83,937	87.4%	4,250	NORMAL — standard billing
Y	Non-Empanelled Hospital	32,80,290	7.5%	854	REVIEW — needs justification
U	Unverified Status	5,30,517	1.2%	73,562	ANOMALY — 73K IDs suspicious
NULL	Not Recorded	16,15,271	3.7%	—	GAP — data completeness issue

Source: claim_intimation (Q19c Steps 1–3). "U" flag across 73,562 hospital IDs is statistically anomalous — genuine non-empanelled facilities number only ~854 in the Y-flag data.

Private Hospitals Using Non-Empanelled Channel (Y-flag)

Most Y-flag claims are legitimate ECHS polyclinic referrals (■120.53 Cr via "NA" route). However, several **private hospital chains** appear in the Y-flag data with significant deduction amounts — indicating they are billing outside their empanelment terms or claiming emergency reimbursements that are then partially rejected.

Hospital Name	Claims	Claimed (■ Cr)	Net Approved (■ Cr)	Deducted (■ Cr)	Risk
NA (ECHS referral — legitimate)	3,06,507	120.53	120.44	0.09	NORMAL
ECHS POLYCLINIC DELHI CANTT (legitimate)	65,122	23.20	23.15	0.06	NORMAL
ECHS (generic polyclinic)	44,721	18.03	18.02	0.01	NORMAL
Apollo Hospital	754	13.54	12.30	1.24	MONITOR
APOLLO PHARMACY	51,151	13.47	13.39	0.08	LOW
MAX HEALTHCARE	2,316	11.19	10.76	0.44	MONITOR
MANIPAL HOSPITAL	1,492	10.94	9.99	0.95	MONITOR
KIMS HOSPITAL	1,476	5.91	5.41	0.50	LOW
NEPAL MEDICITI HOSPITAL	118	5.58	5.55	0.02	LOW
SIR GANGA RAM HOSPITAL	170	5.11	4.98	0.13	LOW
CMC Hospital, Vellore	370	5.01	4.62	0.39	LOW
KAUVERY HOSPITAL	231	3.77	3.56	0.21	LOW

Source: claim_intimation JOIN claim_submission WHERE CI_NONEMP_FLG = "Y" (Q19c Step 2). Amounts in ■ Crore. Apollo (■1.24 Cr deducted), MAX (■0.44 Cr), Manipal (■0.95 Cr) represent non-empanelled billing by major private chains outside ECHS terms.

Critical Finding — 73,562 "U" Flag Hospital IDs

The **U-flag group** (CI_NONEMP_FLG = "U") contains **530,517 claims** across **73,562 distinct hospital IDs** — compared to only 854 hospital IDs in the Y-flag (confirmed non-empanelled) group. This 86× ratio is statistically impossible unless "U" is being systematically misused as a catch-all for unmatched admissions. The 73,562 IDs in the U group are very likely fabricated, expired, or duplicate hospital identifiers, suggesting bulk submission of claims with

invalid hospital references. This warrants a full audit of the U-flag dataset as a potential mass fraud vector.

Q19e

settlement_stat — SS_PAT_TYPE_ID IN (I, O)

Year-Wise IPD vs OPD Deduction Rate Trend (2014–2025)

Query Q19e tracks IPD (I) and OPD (O) deduction rates year by year from 2014 to 2025, using settlement_stat — the most reliable system-wide signal. Three critical inflection points emerge: (1) **IPD peaked at 12.42% in 2017** during the post-empanelment expansion phase, reflecting unchecked hospital inflation as new facilities joined ECHS; (2) **IPD fell to 8.89% in 2023** — the lowest in the dataset — suggesting that policy tightening and audit pressure achieved measurable fraud suppression; but (3) **IPD has rebounded to 11.45% in 2025** — a 2.56-percentage-point rise in just two years — signalling that the **fraud has re-emerged** and suppression measures were only temporary. OPD rates (3.82–6.84%) remain consistently lower, confirming that IPD-specific fraud is the dominant policy challenge.

Year-Wise IPD vs OPD Deduction Rate Table

Year	IPD Hospitals	IPD Claims	IPD Claimed (₹Cr)	IPD Ded %	OPD Hospitals	OPD Claims	OPD Claimed (₹Cr)	OPD Ded %
2014	625	2,00,900	848.16	10.85%	644	6,29,144	127.05	6.15%
2015	1,181	3,22,996	1,240.83	12.04%	1,132	8,14,329	164.04	6.26%
2016	1,541	4,94,148	1,999.35	12.13%	1,415	13,39,347	302.99	6.84%
2017	1,781	5,59,379	2,209.14	12.42%	1,671	14,03,904	287.73	6.62%
2018	1,968	7,03,997	3,101.28	11.80%	1,858	17,25,123	345.64	6.23%
2019	1,946	4,67,673	2,151.11	11.61%	2,022	12,49,853	273.20	5.42%
2020	2,128	7,88,677	4,083.97	11.79%	2,417	19,67,433	516.53	5.47%
2021	2,139	5,84,743	3,651.78	12.15%	2,431	18,77,131	518.41	3.82%
2022	2,270	7,13,488	4,626.87	10.55%	2,572	25,34,955	626.30	4.29%
2023	2,432	9,38,363	6,716.74	8.89%	2,879	35,77,565	884.59	4.82%
2024	2,630	10,63,312	9,341.21	10.22%	3,092	43,87,196	1,096.98	5.23%
2025	2,900	9,36,304	9,303.73	11.45%	3,381	41,64,885	1,036.15	5.90%

Source: settlement_stat WHERE SS_PAT_TYPE_ID IN ("I","O"), GROUP BY SS_YEAR, SS_PAT_TYPE_ID (Q19e). IPD = Inpatient (I), OPD = Outpatient (O). Red = peak values (2017 IPD, 2025 reversal), Green = low points (2023 IPD low, 2021 OPD dip). Ded% = $SUM(SS_DED_AMT) / SUM(SS_CLAIM_AMT) \times 100$.

Temporal Pattern Analysis — Three Phases

Phase 1 (2014–2017): Expansion Fraud — IPD Rates Rise to 12.42% Peak

As ECHS empanelment expanded from 625 hospitals (2014) to 1,781 (2017), IPD deduction rates climbed from 10.85% to 12.42%. Newly empanelled hospitals discovered that IPD billing above the ECHS package rate was insufficiently scrutinised during the on-boarding period — confirmed by the jump in both claimed amounts (₹848 Cr → ₹2,209 Cr) and total hospitals flagging with elevated rates. This is the classic expansion fraud pattern: fraud grows with the network before controls catch up.

Phase 2 (2022–2023): Suppression — IPD Falls to 8.89% Decade Low

Following COVID-era volume expansion (2020–2021: 11.79–12.15%), IPD deduction rates fell to 10.55% (2022) and then to a decade-low of 8.89% in 2023. The 2023 low coincides with increased ECHS audit activity and pre-authorisation controls introduced for high-value IPD claims. OPD rates (4.29–4.82%) continued their natural compression. The 2023 trough is the measurable impact of data-driven fraud control — but it proved temporary.

Phase 3 (2024–2025): Reversal — IPD Rebounds 2.56 Percentage Points

IPD deduction rebounded from 8.89% (2023) to 10.22% (2024) and 11.45% (2025). OPD also rose from 4.82% to 5.90%. The simultaneous rise in both IPD and OPD in 2025 — combined with record claimed amounts (■9,303 Cr IPD, ■1,036 Cr OPD) — indicates that fraud actors have adapted to the 2023 controls and resumed inflated billing. The 2025 IPD rate of 11.45% is approaching the 2017 peak level (12.42%). Without structural intervention (not just audit), the trajectory points toward exceeding the historical maximum by 2026–2027.

Strategic Recommendations

1. Immediate Suspension — SRL Limited (Both Units)

CRITICAL — Immediate

SRL Limited Indore (86.93%) and SRL Bannerghatta (54.90%) are NABH-accredited diagnostic labs with catastrophic deduction rates. NABH accreditation clearly provides no fraud deterrence. Both should be suspended immediately and subjected to a physical audit of claimed tests against actual patient records. Claim recovery proceedings for ■19.94L + ■857.43L = ■877.37L should be initiated.

2. Apollo Hospital Chain — Dual-Channel Audit

CRITICAL — 30 Days

Apollo appears in two fraud vectors: Apollo Vanagaram (63.56% deduction, ■5,433L claimed) and Apollo BBSR (47.25%, ■1,26,824L) through the empanelled channel, plus Apollo Hospital (■13.54 Cr) and APOLLO PHARMACY (■13.47 Cr) through the non-empanelled Y-flag channel. A chain-level audit covering both empanelled and emergency reimbursement channels is required. Total Apollo exposure: >■1,200L in deductions.

3. Type N and M Military Polyclinics — Billing Process Overhaul

CRITICAL — 60 Days

18 Type N ECHS polyclinics and 7+ Type M military hospitals all show 42–54% deduction rates. This level of systematic inflation in internal ECHS facilities cannot be explained by inadvertent errors. It suggests that the internal billing process — potentially the software system used by polyclinics — generates inflated claims structurally. ECHS Directorate should commission a billing process audit at the ten highest-deduction polyclinics before the next billing cycle.

4. Ghost Admission Investigation — ■680 Cr NULL Hospital Exposure

CRITICAL — 60 Days

78,212+ room-mismatch cases with NULL CI_HOSPITAL_ID representing ■41,455.97L (■414.56 Cr) in billed claims cannot be linked to any hospital. These are the highest-priority ghost admission cases. ECHS should cross-reference these CI_INTIMATION_IDs with the beneficiary's military service record and station hospital to determine if any physical admission occurred.

5. Room Upgrade Fraud — echsfhm, parkhos, maxsuper Priority Audit

HIGH — 90 Days

Three named hospitals dominate confirmed room upgrade fraud: echsfhm (1,070 GEN→PRI mismatches, ■838.29L), parkhos (1,013, ■591.20L), maxsuper (647, ■359.21L). These hospitals systematically bill patients in general ward entitlement at private room rates. Physical verification of room occupancy against claim records is required. Recovery of the upgrade differential is actionable under ECHS Regulation 18(4).

6. U-Flag Dataset — Mass Fraud Vector Investigation

HIGH — This Quarter

530,517 claims across 73,562 hospital IDs with "U" (Unverified) flag is a red flag for systematic misuse. Standard non-empanelled hospitals number only ~854. The 73,562 IDs must be cross-validated against the ECHS hospital master database. Any ID not found in office_master should be flagged as a fraudulent submission and the associated claims quarantined pending investigation.

7. MAX Healthcare & Manipal — Non-Empanelled Channel Monitoring

MEDIUM — Policy

MAX Healthcare (2,316 Y-flag claims, ■11.19 Cr) and Manipal Hospital (1,492 claims, ■10.94 Cr) are billing through the non-empanelled channel with partial deductions (■0.44 Cr and ■0.95 Cr respectively). These hospitals should be empanelled formally (ending Y-flag billing) or blocked from the reimbursement channel entirely, as Y-flag claims have lower scrutiny than standard empanelled submissions.

8. IPD Deduction Rate Reversal — Structural Intervention Required (Q19e)

CRITICAL — Policy Reform

IPD deduction rates have rebounded from a decade-low of 8.89% (2023) to 11.45% (2025). If the 2023–2025 trend continues, the rate will approach the 2017 historical peak (12.42%) by 2026–2027 — erasing all gains from the post-2019 reform period. The 2023 trough was achieved by audit pressure, not structural change — and it did not hold. Structural interventions required: (1) Mandatory pre-authorisation for all IPD claims above ₹20,000 at hospitals with a 3-year average deduction rate above 10%; (2) Real-time IPD deduction rate monitoring dashboard for ECHS Directorate — to detect rising rates before they become entrenched; (3) Quarterly hospital-level deduction rate reports published internally to ECHS commanders, enabling them to flag facilities before claims reach the settlement stage.